



Anaphylaxis

Call ambulance

Check

Airway	open airway (consider intubation if needed)
Breathing	assess effectiveness of ventilation + give O2 10L/min (aiming for > 94%)
Circulation	check pulse, BP and perfusion, fix large bore IV cannula
Disability	assess consciousness level by GCS
Exposure	check skin changes (urticarial), insect bite site, facial or lip swelling (angioedema)

Start fluids 0.9% saline for:

- Adults: **500ml-1L in 5-10 min**
- Children: **20 ml/kg**

There are no absolute contraindications to Epinephrine in the setting of anaphylaxis

- **Adrenaline IM 1:1000 is the first and main treatment for anaphylaxis** → Anterolateral aspect – middle 1/3 of the thigh Repeat after 5 min if no improvement..
 - If patient is on Beta blocker and no response to Adrenaline → **Give Glucagon** (1-5 mg slowly IV bolus over 5 min)
 - If bronchospasm not responsive to Adrenaline → **Give inhaled Salbutamol**
- **Oral Antihistamine** Chlorphenamine (Peritone) after stabilizing patient especially if presented with skin symptoms e.g. urticaria and/or angioedema (non sedating IV antihistamine e.g. Cetirizine are preferred).
- **Antihistamines & Hydrocortisone are consideration Adjunctive agents.**

Age	IM Adrenaline (1:1000)	IM or slow IV Hydrocortisone	IM or slow IV Chlorphenamine (Peritone)
> 12 Yr.	0.5 ml	200 mg	10 mg
6 – 12 Yr.	0.3 ml	100 mg	5 mg
6 Mon – 6 Yr.	0.15 ml	50 mg	2.5 mg